This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Salbutamol 100mcg MDI inhaler and Prednisolone 5mg tablets for the treatment of Asthma Rescue

Patient Group Direction, version 009, issued 30 September 2026. Supersedes Version 008, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Asthma Rescue

Asthma Overview

- **Definition:** Chronic inflammatory condition of the airways presenting with recurrent episodes of wheezing, breathlessness, chest tightness, and cough.
- **Diagnosis:** Based on clinical history and pulmonary function testing (spirometry); must be confirmed prior to this PGD.

Acute Exacerbation Assessment

- **Features of acute exacerbation:** Worsening breathlessness, wheezing, cough, chest tightness not relieved by usual reliever therapy.
- **Severity assessment:** Mild-moderate exacerbation (able to complete sentences), versus severe (unable to speak full sentences, distress, low oxygen saturation).
- **Red flags:** Silent chest, exhaustion, confusion, or hypoxia warrant emergency referral.

Rescue Treatment

- **Short-acting beta-2 agonist (SABA):** First-line for acute symptom relief (salbutamol).
- **Oral corticosteroids: For acute exacerbations to reduce airway inflammation (prednisolone 40mg once daily for 5 days).**
- **Response monitoring:** Assess response at 15-30 minutes; if insufficient relief, refer for emergency assessment.
- **Salbutamol limits under this PGD: maximum 8 puffs in 24 hours; relief needed more often than every 4 hours, or on most days, is a same-day GP referral; in an acute attack up to 10 puffs through a spacer, and no relief after 10 puffs is 999.**

Patient Group Direction

for the supply of

Salbutamol 100mcg MDI inhaler

for the treatment of

Asthma Rescue

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do

	so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute symptom relief in adults with diagnosed asthma presenting with acute exacerbation (breathlessness, wheezing, chest tightness)
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of asthma, DOCUMENTED: GP record, a repeat prescription for an asthma inhaler, or an asthma action plan. Previous inhaler use on its own is not confirmation. Current preventer (inhaled corticosteroid) therapy asked about and recorded. A patient with no preventer, or who has needed more than one rescue course in the last 12 months, is referred to the GP for review; no more than one rescue course in 12 months is supplied under this PGD. Acute exacerbation with symptoms of bronchospasm (wheezing, breathlessness, chest tightness) Capable of using inhaler device or willing to use spacer Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to salbutamol or other beta-2 agonists Acute severe or life-threatening asthma: any of unable to complete sentences in one breath, respiratory rate 25/min or more, heart rate 110/min or more, PEF 50% of best or predicted or below (33 to 50% is acute severe; below 33% is life-threatening), SpO2 below 92%, silent chest, cyanosis, exhaustion, confusion or poor respiratory effort. Refer for emergency assessment (999 where life-threatening). Do not supply. Pulse oximetry, respiratory rate and pulse must be measured and recorded before any supply; where a peak flow meter is available, record PEF. If any observation is missing, do not supply. First presentation suggestive of asthma without prior diagnosis - refer for assessment
Cautions	Cardiovascular disease: beta-2 agonists can increase heart rate and

	blood pressure Diabetes mellitus: hyperglycaemia can occur. Advise a patient who self-monitors to check blood glucose more often during the exacerbation and to report readings higher than usual to the GP or diabetes team. Hyperthyroidism: beta-2 agonists can worsen symptoms Hypokalaemia: may be worsened by beta-2 agonists, particularly with diuretics, corticosteroids or theophylline. Potassium cannot be measured in the pharmacy; where the patient is known to have low potassium, inform the GP so that it can be checked. Pregnancy: salbutamol is safe but ensure appropriate informed consent
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Salbutamol 100mcg metered-dose inhaler (MDI)
Legal category	POM
Storage	Store below 25°C. Keep away from heat and direct sunlight. Shake well before each use.
Route/method of administration	Inhalation via MDI, with or without spacer
Dose and frequency	2-4 puffs as required for acute symptom relief Initial dose: 2-4 puffs inhaled immediately for acute exacerbation Use spacer device in less experienced patients to optimise drug delivery May repeat after 15-30 minutes if inadequate response Maximum 8 puffs in 24 hours under this PGD. A need for more than this, for relief more often than every 4 hours, or for the reliever on most days, is uncontrolled asthma: same-day GP or urgent care referral. In an acute attack, up to 10 puffs through a spacer, one puff at a time, may be taken while help is sought; this acute-attack step is not barred by the 24 hour maximum. Ten puffs through a spacer with no relief is an emergency: call 999.
Quantity to be administered and/or supplied	1 MDI inhaler (200 doses)

Maximum or minimum treatment period	As needed during the acute exacerbation, within the maximum of 8 puffs in 24 hours. One inhaler (200 actuations) per supply; no more than one rescue course in any 12 months under this PGD (see inclusion criteria).
Adverse effects	Common: tremor (fine tremor of hands), headache, nervousness, tachycardia Uncommon: palpitations, muscle cramps, anxiety Rare: hypersensitivity reactions, arrhythmias, myocardial ischaemia

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the inhaler technique sheet provided to ensure optimal drug delivery. If using a MDI without a spacer, coordinate inhalation with actuation. If symptoms do not improve within 15-30 minutes of salbutamol use, seek emergency medical attention.

	Do not use more than 8 puffs in 24 hours. If you need it more often than every 4 hours, or on most days, see your GP the same day. In an asthma attack, take up to ten puffs through a spacer, one puff at a time, while help is sought; if ten puffs through a spacer give no relief, call 999. Seek immediate medical attention if experiencing severe breathlessness, chest pain, confusion, or exhaustion during the exacerbation. Review your asthma action plan and triggers with your GP after recovery. Ensure your asthma maintenance therapy is optimised to prevent future exacerbations.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Asthma: diagnosis and management
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply of

Prednisolone 5mg tablets

for the treatment of

Asthma Rescue

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of acute asthma exacerbation to reduce airway inflammation and improve bronchospasm
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of asthma MODERATE exacerbation with incomplete response to salbutamol: able to complete sentences in one breath, PEF over 50% of best or predicted where measurable, SpO2 92% or above, respiratory rate below 25, heart rate below 110, and no acute severe or life-threatening feature. Any acute severe or life-threatening feature is an emergency referral, not a prednisolone supply. Able to take oral medication No contraindications to corticosteroid use Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to prednisolone or other corticosteroids Acute severe or life-threatening asthma: any of unable to complete sentences in one breath, respiratory rate 25/min or more, heart rate 110/min or more, PEF 50% of best or predicted or below (33 to 50% is acute severe; below 33% is life-threatening), SpO2 below 92%, silent chest, cyanosis, exhaustion, confusion or poor respiratory effort. Refer for emergency assessment (999 where life-threatening). Do not supply. Pulse oximetry, respiratory rate and pulse must be measured and recorded before any supply; where a peak flow meter is available, record PEF. If any observation is missing, do not supply. First presentation suggestive of asthma without a prior diagnosis. Refer for assessment. Systemic infection (unless treated with appropriate antimicrobials) Vaccination with live vaccines during treatment Severe hepatic dysfunction Uncontrolled hypertension or unstable cardiac disease. Do not supply under this PGD; refer to the GP or urgent care.
Cautions	Diabetes mellitus: corticosteroids raise blood glucose. Advise the patient to check glucose more often during the course and to contact the GP or diabetes team about readings higher than usual. Diabetes medicines are not adjusted under this PGD. Osteoporosis or risk factors: short-term treatment risk is low but inform patient

	Peptic ulcer disease or GI upset: take with food. Gastroprotection is not supplied under this PGD; where there is a history of peptic ulcer or gastrointestinal bleeding, inform the GP. Psychiatric history: corticosteroids can cause mood changes, insomnia, or exacerbate existing conditions Renal impairment: generally safe for short-term use Hypertension: may worsen; monitor blood pressure Infections: can mask symptoms; ensure appropriate investigation before use Pregnancy and breastfeeding: generally safe for acute use; discuss benefits/risks
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Prednisolone 5mg tablets
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from light and moisture.
Route/method of administration	Oral
Dose and frequency	40mg once daily, taken as a single dose in the morning, for 5 days. This is the only dose and course under this PGD. May be taken with food to reduce GI upset Prednisolone 5mg tablets: 40mg daily is EIGHT tablets a day. Eight tablets is one day of treatment, not a course.
Quantity to be administered and/or supplied	40 tablets of 5mg (40mg daily, eight tablets a day, for 5 days). No other quantity is supplied under this PGD. Check the tablet count against the dose before supply.
Maximum or minimum treatment period	5 days
Adverse effects	Very common: insomnia, mood changes (euphoria, irritability, anxiety), increased appetite Common: GI upset (dyspepsia, nausea), increased sweating, headache, dizziness

	Uncommon: acne, facial flushing, restlessness, hypertension, hyperglycaemia, immunosuppression Rare: adrenal suppression (with prolonged use), osteoporosis, avascular necrosis, psychiatric disturbances, opportunistic infections

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the inhaler technique sheet provided to ensure optimal drug delivery. If using a MDI without a spacer, coordinate inhalation with actuation. If symptoms do not improve within 15-30 minutes of salbutamol use, seek emergency medical attention. Do not use more than 8 puffs in 24 hours. If you need it more often than every 4 hours, or on most days, see your GP the same day. In an asthma attack, take up to ten puffs through a spacer, one puff at a time, while help is sought; if ten puffs through a spacer give no relief, call 999.

	Take the full 5 day course of prednisolone (eight 5mg tablets once each morning), even if symptoms improve. Take prednisolone with food if it causes stomach upset. Do not stop prednisolone abruptly; follow the course as directed. If diabetic, monitor blood glucose more frequently as prednisolone may raise levels; inform your GP. If taking other medications, inform your healthcare provider of steroid use. Seek immediate medical attention if experiencing severe breathlessness, chest pain, confusion, or exhaustion during the exacerbation. Review your asthma action plan and triggers with your GP after recovery. Ensure your asthma maintenance therapy is optimised to prevent future exacerbations.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v009

Issued: 30 September 2026

Supersedes: Version 008, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v008

Issued: 14 September 2026

Supersedes: Version 007, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v007

Issued: 11 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 37: the salbutamol arm now carries the COPD limits: maximum 8 puffs in 24 hours; relief needed more often than every 4 hours or on most days is a same-day GP or urgent care referral; up to 10 puffs through a spacer is kept as the acute-attack step and 10 puffs without relief is 999; stated in the guidance summary, dose row, treatment period and follow-up advice.

Decision 38: prednisolone fixed at 40mg once daily as a single morning dose for 5 days, 40 tablets of 5mg, as the only regimen; the 50mg option, the 7 day course and the 70 tablet maximum are removed from the guidance summary, dose row, quantity row, treatment period and follow-up advice.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm headings corrected from administration to supply; both arms are supply.

PEF exclusion in both arms now reads 50% of best or predicted or below, so that a PEF below 33% is excluded in terms and a PEF of exactly 50% is excluded by the inclusion and the exclusion alike.

The relative exclusion for uncontrolled hypertension or cardiac disease in the prednisolone arm is a firm exclusion with a referral; a PGD cannot carry a discuss-risk-benefit exclusion.

Cautions that asked the pharmacy to monitor potassium or blood glucose, adjust diabetes medication or consider gastroprotection are reworded as actions a pharmacy can take.

Prednisolone dose fixed as a single morning dose of 40mg to 50mg daily; divided doses removed.

Salbutamol arm follow-up no longer tells the patient to take a course of prednisolone.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: The prednisolone arm stated "for 5-7 day course: 8-10 tablets of 5mg" and, two lines later, "up to 42 tablets". Eight to ten tablets is a single day at 40 to 50mg. The quantity is now stated from the dose: 40 tablets for 40mg daily for 5 days, to a maximum of 70 for 50mg daily for 7 days. The emergency red flags in the salbutamol arm, hypoxia, silent chest, exhaustion, confusion and first presentation, were not in the prednisolone arm's exclusions. They are now. This incorporates and withdraws the correction notice of 7 September 2026. The salbutamol arm is unchanged.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The expiry cells read 31 October 2026.

Exclusion criteria, both arms: the previous version excluded a severe attack only where SpO2 was below 92%, which is the life-threatening threshold. The BTS/SIGN acute severe criteria (cannot complete sentences, RR 25 or more, HR 110 or more, PEF 33 to 50%) now exclude, and pulse oximetry, respiratory rate and pulse must be measured and recorded before any supply.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

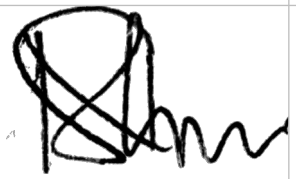
Changes in this version:

Salbutamol arm: 'previous inhaler use' no longer counts as a confirmed diagnosis, which contradicted the guidance summary and the exclusion for first presentation. A documented diagnosis is required; preventer therapy is recorded; a patient with no preventer or more than one rescue course in 12 months is referred to the GP for review.

Prednisolone arm: 'inadequately responsive to SABA alone' was both the entry criterion here and an emergency-referral trigger in the guidance summary. The arm now defines the moderate exacerbation it treats (able to complete sentences, PEF over 50%, SpO2 92% or above, RR under 25, HR under 110, no severe feature); anything meeting the acute severe criteria is referred.

Signed on behalf of Get Real Health

Version v009, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.